

MEDICAL CONSULTATION REPORT

PATIENT INFORMATION

Name:	John Smith
Date of Birth:	05/15/1978
Gender:	Male
Medical Record Number:	MRN-12345678
Date of Visit:	06/10/2023

CHIEF COMPLAINT

Patient presents with persistent fatigue, joint pain, and morning stiffness for the past 3 months. Reports occasional fever and weight loss of approximately 10 pounds over the same period.

HISTORY OF PRESENT ILLNESS

The patient is a 45-year-old male with a history of hypertension and hyperlipidemia who presents with symptoms consistent with possible rheumatoid arthritis. Symptoms began approximately 3 months ago with morning stiffness lasting more than 1 hour and symmetric joint pain affecting the wrists, hands, and knees. The patient denies any recent trauma or infection that might have triggered these symptoms.

PAST MEDICAL HISTORY

- Hypertension (diagnosed 5 years ago)
- Hyperlipidemia (diagnosed 3 years ago)
- Type 2 Diabetes Mellitus (diagnosed 2 years ago)
- Obstructive Sleep Apnea (diagnosed 1 year ago)
- History of Coronary Artery Disease (diagnosed 1 year ago)

CURRENT MEDICATIONS

- Lisinopril 10mg daily for hypertension
- Atorvastatin 20mg daily for hyperlipidemia

- Metformin 500mg twice daily for diabetes
- Aspirin 81mg daily for cardiovascular protection
- CPAP therapy for sleep apnea

FAMILY HISTORY

- Father: Deceased at age 65 from myocardial infarction, history of hypertension and diabetes
- Mother: Alive, age 70, with rheumatoid arthritis and osteoporosis
- Sister: Alive, age 42, with systemic lupus erythematosus
- Brother: Alive, age 38, with psoriasis

SOCIAL HISTORY

- Occupation: Software engineer
- Tobacco: Former smoker, quit 10 years ago, 20 pack-years
- Alcohol: Occasional use, 1-2 drinks per week
- Exercise: Sedentary lifestyle due to recent symptoms

PHYSICAL EXAMINATION

Vital Signs:

- Temperature: 99.8°F
- Blood Pressure: 138/82 mmHg
- Heart Rate: 82 bpm
- Respiratory Rate: 16 breaths/min
- Weight: 185 lbs (decreased from 195 lbs 3 months ago)

General: Well-appearing male in no acute distress

HEENT: Normal

Cardiovascular: Regular rate and rhythm, no murmurs

Respiratory: Clear to auscultation bilaterally

Abdomen: Soft, non-tender, non-distended

Musculoskeletal: Symmetric swelling and tenderness of MCP and PIP joints of both hands, wrists, and knees. Morning stiffness noted.

Skin: No rash noted

Neurological: Intact

LABORATORY RESULTS

Test	Result	Reference Range	Units
WBC	8.2	4.5-11.0	K/uL

Hgb	13.5	13.5-17.5	g/dL
Hct	41	41-50	%
Platelets	245	150-450	K/uL
ESR	45	0-20	mm/hr
CRP	2.8	<0.5	mg/dL
RF	60	<14	IU/mL
Anti-CCP	Positive	Negative	
ANA	1:160	<1:40	
Creatinine	1.1	0.7-1.3	mg/dL
ALT	28	7-56	U/L
AST	24	10-40	U/L
HbA1c	6.8	4.0-5.6	%
LDL	98	<100	mg/dL
HDL	42	>40	mg/dL
Triglycerides	150	<150	mg/dL

ASSESSMENT

1. Rheumatoid Arthritis - New diagnosis based on clinical presentation, positive RF and anti-CCP antibodies, and elevated inflammatory markers.
2. Hypertension - Well-controlled on current medication.
3. Type 2 Diabetes Mellitus - Suboptimal control based on HbA1c of 6.8%.
4. Hyperlipidemia - Well-controlled on statin therapy.
5. Obstructive Sleep Apnea - Compliant with CPAP therapy.
6. Coronary Artery Disease - Stable on current therapy.

PLAN

1. Rheumatoid Arthritis:
 - Start Methotrexate 15mg weekly with folic acid 1mg daily
 - Prednisone 10mg daily for 2 weeks, then taper
 - Refer to Rheumatology for specialized care
 - Physical therapy referral for joint protection and exercise program
2. Diabetes:
 - Increase Metformin to 1000mg twice daily
 - Continue current diet and exercise recommendations
 - Follow up in 3 months for HbA1c check
3. Continue current medications for hypertension, hyperlipidemia, and cardiovascular protection.

4. Follow up in 2 weeks to assess response to RA treatment and adjust medications as needed.